



Laboratory Investigation Report

Patient Name	: Geeta	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 31 Y 6 M 7 D /F	OP/IP No	: OP/96397
Max ID/Mobile	: 334940/8752005200	Collection Date/Time	: 11/Aug/2021 09:16AM
Lab ID	: 1266082100249	Receiving Date	: 11/Aug/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 12/Aug/2021

Serology Elisa

IgE (Immunoglobulin-E)*, Serum

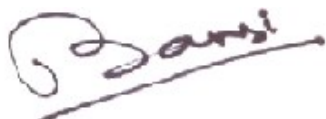
FEIA

Date	11/Aug/2021 09:16AM	Unit	Bio Ref Interval
Immunoglobulin-IgE	40.2	kUA/L	0-64

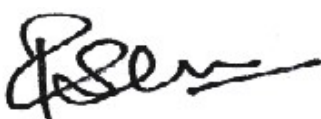
Comment Total IgE is an in vitro test system for the quantitative measurement of circulating total IgE in human serum or plasma. It is intended for in vitro diagnostic use as an aid in the clinical diagnosis of IgE mediated allergic disorders in conjunction with other clinical findings, and is to be used in clinical laboratories. A definite clinical diagnosis should not be made as a result of single test only, but should be made by taking into account clinical history and other laboratory findings.

Kindly correlate with clinical findings

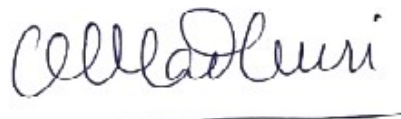
*** End Of Report ***



Dr. Bansidhar Tarai, M.D.
Associate Director
Microbiology & Molecular Diagnostics



Dr. Poornima Sen, M.D.
Consultant - Microbiology



Dr. Madhuri Somani, M.D. , DNB
Consultant - Microbiology



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SIN No: B2B840239, Test Performed at : 910 - Max Hospital - Saket M S S H, Press Enclave Road, Mandir Marg, Saket, New Delhi, Delhi 110017

Booking Centre : 1534 - Arctern Healthcare Noida, 2nd Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, 8752

Max Lab - A Division of Max Healthcare Institute Ltd.
Max Super Specialty Hospital, Saket (West Block)
The : 1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.

CIN No.: L72200MH2001PLC322854

www.maxlab.co.in

Conditions of Reporting: 1. The tests are carried out in the lab with the presumption that the specimen belongs to the patient name as identified in the bill/test request form. 2. The test results relate specifically to the sample received in the lab and are presumed to have been generated and transported per specific instructions given by the physicians/laboratory. 3. The reported results are for the information and interpretation by the referring doctor only. 4. Some tests are referred to other laboratories to provide a wider test menu to the customer. 5. Max Healthcare shall in no event be liable for accidental damages loss, or destruction of specimen which is not attributable to any direct and mala fide act or omission of Max Healthcare or its employees. Liability of Max Healthcare for deficiency of services, or other errors and omissions shall be limited to fee paid by the patient for the relevant laboratory services.



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Clinical Biochemistry

Test Name	Result	Unit	Bio Ref Interval
IgA (Immunoglobulin-A)*, Serum			
Immunoglobulin IgA Immunoturbidimetric	297.42	mg/dL	70 - 400

Interpretation

Polyclonal-increased seen in chronic inflammatory disorders, infectious processes and in association with tumors, rheumatoid arthritis, inflammatory bowel disease, ankylosing spondylitis, mixed connective tissue disease.

Monoclonal-increased seen in multiple myeloma, solitary plasmacytoma, α -heavy chain disease, monoclonal gammopathy of undetermined significance (MGUS), Lymphoma and Chronic Lymphocytic leukemia.

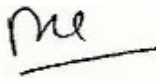
Decreased in primary immunodeficiencies associated with infectious, autoimmune and allergic complications, gastric carcinoma and lymphoma.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Poonam. S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology



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Lab ID	: 1266082100249	Receiving Date	: 11/Aug/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 14/Aug/2021

Test Name	Result	Unit	Bio Ref Interval
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MTHFR Gene Mutation PCR (L)*, EDTA

Real Time PCR

MTHFR (Methyl Tetrahydrofolate Reductase) GENE MUTATION

C677T	Not Detected
A1298C	Heterozygous mutation detected

Ref Range

Result	Comments
Homozygous mutation detected	Both copies of the gene carry mutation.
Heterozygous mutation detected	One copy of the gene carries mutation.
Not Detected	Mutation not detected.

Note

1. This assay detects the mutation C677T and A1298C in MTHFR.
2. This is an in-house developed assay
3. Test conducted on Whole blood
4. Genetic counseling available.

Comments

Homozygous mutation for MTHFR (Methyltetrahydrofolate reductase) is associated with hyperhomocysteinemia which is an independent risk factor for Stroke, Myocardial infarction, Peripheral arterial disease and venous thrombosis. Indian studies suggest that heterozygosity for MTHFR C 677T is also associated with elevated homocysteine levels. MTHFR C677T or A1298C carriers are not at increased risk for thrombosis in the absence of hyperhomocysteinemia. Homozygous MTHFR C 677T or A1298C carriers are at increased risk for hyperhomocysteinemia if they become deficient in vitamins B 6, B12 or folic acid. Hyperhomocysteinemia is a relatively weak risk factor for both venous thromboembolism and arterial thrombosis. According to ACMG guidelines there is growing evidence that MTHFR polymorphism testing has minimal clinical utility and, therefore should not be ordered as a part of a routine evaluation for thrombophilia.



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Test Name	Outsourced Result	Unit	Bio Ref Interval
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Note : Outsourced Test

Kindly correlate with clinical findings

*** End Of Report ***



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Hematology Special

Test Name	Result	Unit	Bio Ref Interval
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Natural Killer Cells, Flow Cytometry, EDTA

Flow Cytometry Number	IM 123/2021
Total Leucocytes	4500 cells/μl
Lymphocyte percentage	33.1 %
NK cells percentage:	2.78 % of gated lymphocytes
NK cells absolute count	41.4 cell/ul
(Expected reference range 67 - 1134 cells/μl)	

Lupus Anticoagulant, Sodium Citrate

Silica Clotting Time

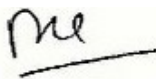
SCT Screen	34.60	Sec	30 - 49
SCT Screen ratio	0.88		
SCT Confirm	28.10	Sec	26.6 - 38.8
SCT Confirm ratio	0.86		
SCT Screen : Confirm ratio	1.02		< 1.20
Interpretation	No Lupus Like Anticoagulant Present		

Kindly correlate with clinical findings

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